

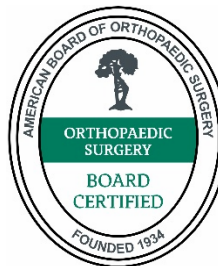
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ASSH | American Society
for Surgery of the Hand



RE: Martinez, Lelia
Date of Birth: 11/09/1948
Date of Injury: 02/29/2024
Date of Exam: 04/04/2024
Parts of Body: Right shoulder, Right wrist, Right hand

INITIAL ORTHOPAEDIC SPECIALTY SURGEON/PHYSICIAN EXAMINATION, EVALUATION AND MANAGEMENT REPORT

To Whom It May Concern:

Ms. Lelia Martinez was seen in the office today for initial orthopedic specialty surgeon/physician examination, evaluation and management.

A translator was not required. Today's visit was conducted in both English and Spanish by both the examiner and the patient.

The following is a dictated narrative report based on pre-examination interview, specialty examination, review of diagnostic/imaging studies and review of medical records. Included in this report are findings, diagnosis/impression, prognosis, work status and further evaluation and treatment recommendations with respect to this injury.

History of Present Injury/Illness

Ms. Martinez is a 75-year-old right-hand-dominant female who was involved in a motor vehicle accident on 02/29/2024 while in her 2014 Hyundai vehicle. She was involved in a hit-and-run accident when she was struck/hit from behind and she reports getting dizzy and saw a big white truck behind her. She was the driver of her vehicle wearing her seat belt. Airbags did not deploy. She reports she did have loss of consciousness, but had no loss of recall or amnesia to the accident, but felt she was dizzy. Her vehicle was totaled. Since then, she has had right shoulder and right hand pain, numbness and tingling of her right middle finger and right ring finger. She also reports discomfort in her upper back. She was not seen at a hospital nor urgent care after the accident. She has been seen by Sanjay Ghosh, M.D. on 02/29/2024. She does report having been seen at Sharp Emergency Room sometime after the accident and has had ongoing neck pain with radiating pain, numbness and weakness of the right upper extremity. She was seen by Sanjay Ghosh, M.D. on 02/29/2024, who referred her to the Orthopedic service for right shoulder pain with possible injury.

Past History of Similar Illness/Injury

The patient denies any significant past medical history of injuries or disabilities to the right shoulder pain and any right hand or right upper extremity numbness or tingling prior to initial date of current injury of 02/29/2024.

Current Complaints

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She reports ongoing right shoulder, right elbow, right hand bothersome body parts and symptoms with numbness and tingling. She has 8/10 pain on average over the last week and 8/10 overall disability/dysfunction. Pain quality is sharp and stabbing with associated symptoms of weakness, numbness, tingling and decreased range of motion. Timing of pain is intermittent and occurs in the mornings and with activities including twisting, turning, overhead reaching, physical therapy, throwing and lifting. She feels better with heat and medications. She has had 2 sessions of physical therapy at Spine and Sports so far predominantly focused on her neck. She feels worse while sleeping and in the morning and with twisting, turning, reaching and lifting. She reports no specific nocturnal disturbance related to numbness and tingling in her right hand.

Medical Treatment to Date

<NOT DICTATED>

Activities of Daily Living (ADLs)

She reports having difficulty dressing herself and cutting food and making meals and writing notes and rising from a chair and has difficulty sleeping intermittently and it hurts to open car doors.

Past Medical History

Hypertension and gastroesophageal reflux disease.

Past Surgical History

Ectopic pregnancy related surgery.

Medications

Omeprazole and amlodipine.

Allergies

No known drug allergies.

Family History

Noncontributory.

Social History

She denies use of tobacco, alcohol or illicit drugs. She is single. She has a 40-year-old daughter. She is a retired CNA as of 2010. She lives in San Diego.

Review of Systems

A complete review of systems was performed by way of review of the patient intake forms. The review of systems is negative for a 14-point review of systems aside from what is detailed above in the History of Present Injury/Illness.

Physical Examination

General:

In general, Ms. Martinez is a well-nourished, well-developed female of average build and has no gross deformities and is well-kempt and pleasant and cooperative with today's interview and physical exam.

Cardiovascular:

Cardiovascular wise, there are 2+ distal radial pulses with warm well perfused digits and brisk capillary refill throughout all tested distal extremities.

Lymphatics:

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Lymphatic wise, there are no noted areas of epitrochlear or axillary lymphadenopathy.

Neurologic and Psychologic:

Neurologic and psychologic testing show intact gross coordination with sensation intact to light touch throughout bilateral hands and upper extremities aside from what is detailed below. She is awake, alert and oriented x4 with mood and affect appropriate for the given situation.

Focused Examination:

Focused examination of the left shoulder shows full active range of motion with no provocative maneuvers.

Focused examination of the right shoulder shows active range of motion, forward flexion of 85 degrees, abduction 85 degrees, abduction external rotation 60 degrees, external rotation at the side 40 degrees. She has 4/5 strength with resisted external rotation strength and 5-/5 with resisted internal rotation strength at the side. Positive provocative maneuvers include positive Speed, Jobe, cross body adduction, bear hug maneuver. She has tenderness to palpation at the long head of biceps tendon sheath and AC joint.

Focused examination of the right wrist shows full active range of motion with positive Durkan, Tinel and Phalen maneuvers along the carpal canal.

Focused examination of the right hand shows diminished sensation with light touch testing along the middle finger and ring finger with 5/5 interossei and abductor pollicis brevis strength. She has full composite flexion, extension, and opposition of all digits of the right hand.

Skin:

Skin exam via inspection and palpation shows no other scars, rashes or lesions.

Diagnostic Studies

None were available for review.

Impression/Diagnoses

A discussion was held with the patient in which the diagnoses were explained in detailed layman's terms. It is my opinion that the patient suffers from:

1. Right shoulder: Sprain/strain/contusion, query internal derangement (specifically proximal long head of biceps pathology and/or rotator cuff tear).
2. Right upper extremity: Paresthesias (query peripheral entrapment neuropathy such as carpal tunnel syndrome and/or cervical radiculopathy).

Recommendations

The treating recommendations have been discussed and/or communicated with the patient, as outlined in this transcribed dictation. During this initial examination, evaluation and management encounter, a discussion was held with the patient in which the above diagnoses along with the therapeutic options/alternatives were explained in detailed layman's terms.

She is to be referred for imaging studies including x-rays of the right shoulder with 3 views to be completed an MRI of the right shoulder without contrast to evaluate for internal derangement. She specifically requested Imaging Healthcare Specialists location in Hillcrest given that this is a few blocks away from her home and she has difficulty with transportation at this time. She is also to be referred for electrodiagnostic studies of the right upper extremity to evaluate for cervical radiculopathy and/or peripheral entrapment neuropathy. She is to continue physical therapy at the Spine and Sport location where she is going currently and the physical therapy referral from my perspective is to be for the right

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shoulder and right upper extremity. She is to be dispensed a wrist-hand orthosis of the right wrist to be worn as a nocturnal extension splint as treatment of presumed posttraumatic carpal tunnel syndrome. For pain management, she has Celebrex available in her home. She already has a prescription and has been using that with pain relief and I told her to continue that medication and to supplement with Tylenol. She also uses hot compresses on her right shoulder, which she should continue with as she reports that is being helpful.

The risks, benefits and alternatives to the above treatment plan were explained in detail to the patient who vocalized understanding. All questions were answered to the patient's satisfaction. No guarantees were made with any of my treatment nor are they ever and the patient understood this as well.

Follow-Up Appointment

After x-rays and MRI of the right shoulder and electrodiagnostic studies of the right upper extremity have been completed.

Reimbursable Services

Evaluation & Management (E&M):

My evaluation and management service for date of service 04/04/2024 is a separately reimbursable service.

Medical Records Review:

After examination of the patient and prior to dictating this report, I spent at least 15 minutes reviewing the medical records of the initial and any subsequent treating physician(s)/ physician assistant(s)/healthcare providers, including initial report, patient questionnaires/intake forms, any follow-up treatments and any diagnostic testing performed. These reports have been used in the assistance of formulating the patient's scope of previous treatment as well as current diagnosis and anticipated course of treatment and prognosis. This review of records is a separately reimbursable service.

Dictation Services:

Preparation time for dictation of this report, time spent performing dictation and time spent reviewing/editing/signing the transcribed dictation required an additional 15 minutes of my time. This time spent performing this task is also a separately reimbursable service.

Sincerely,



Kristopher L. Downing, M.D.
Orthopaedic Surgeon
Hand and Upper Extremity Surgeon & Microsurgeon

KLD/TDYNE/bmb.PPU

Electronically signed by Kristopher L. Downing, MD on 04/07/2024 08:37:41 AM